PATIENT NAME : KUNAL KOHLI	**REF. DOCTOR :** SELF	
CODE/NAME & ADDRESS : C000133041 CHECKMED PRIVATE LIMITED SECOND FLOOR, SCO 334-334, SECTOR 35,CHANDIGARH CHANDIGARH 160022 9914256267	ACCESSION NO : **0062YH000953** PATIENT ID : KUNAM08119462 CLIENT PATIENT ID: ABHA NO :	AGE/SEX : 30 Years Male DRAWN : RECEIVED : 14/08/2025 09:11:43 REPORTED : 16/08/2025 13:01:42

Test Report Status **Final**	Results	Biological Reference Interval	Units
ANNUAL HEALTH CHECK-UP PACKAGE			
ECG			
ECG	WITHIN NORMAL LIMITS		
TMT			
STRESS TEST	NEGATIVE		
MEDICAL HISTORY			
RELEVANT PRESENT HISTORY	NOT SIGNIFICANT		
RELEVANT PAST HISTORY	NOT SIGNIFICANT		
RELEVANT PERSONAL HISTORY	MARRIED, 01 CHILD, NON VEG.		
RELEVANT FAMILY HISTORY	MOTHER- HIGH BLOOD PRESSURE, DIABETES, THYROID DISEASE		
OCCUPATIONAL HISTORY	ACCOUNTS.		
HISTORY OF MEDICATIONS	NOT SIGNIFICANTS.		
ANTHROPOMETRIC DATA & BMI			
HEIGHT IN METERS	1.67		mts
WEIGHT IN KGS.	79		Kgs
BMI	28	BMI & Weight Status as follows: Below 18.5: Underweight 18.5 - 24.9: Normal 25.0 - 29.9: Overweight 30.0 and Above: Obese	kg/sqmts
GENERAL EXAMINATION			
MENTAL / EMOTIONAL STATE	NORMAL		
PHYSICAL ATTITUDE	NORMAL		

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ULR No.775000013792634-0062

PATIENT NAME : KUNAL KOHLI		REF. DOCTOR : SELF
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GENERAL APPEARANCE / NUTRITIONAL STATUS	HEALTHY		
BUILT / SKELETAL FRAMEWORK	AVERAGE		
FACIAL APPEARANCE	NORMAL		
SKIN	NORMAL		
UPPER LIMB	NORMAL		
LOWER LIMB	NORMAL		
NECK	NORMAL		
NECK LYMPHATICS / SALIVARY GLANDS	NOT ENLARGED OR TENDER		
THYROID GLAND	NOT ENLARGED		
CAROTID PULSATION	NORMAL		
BREAST (FOR FEMALES)	NORMAL		
TEMPERATURE	NORMAL		
PULSE	59/MIN REGULAR, ALL PERIPHERAL PULSES WELL FELT, NO CAROTID BRUIT		
RESPIRATORY RATE	NORMAL		
CARDIOVASCULAR SYSTEM			
BP	119/69 MM HG (SITTING)		mm/Hg
PERICARDIUM	NORMAL		
APEX BEAT	NORMAL		
HEART SOUNDS	S1, S2 HEARD NORMALLY		
MURMURS	ABSENT		
RESPIRATORY SYSTEM			
SIZE AND SHAPE OF CHEST	NORMAL		
MOVEMENTS OF CHEST	SYMMETRICAL		
BREATH SOUNDS INTENSITY	NORMAL		
BREATH SOUNDS QUALITY	VESICULAR (NORMAL)		
ADDED SOUNDS	ABSENT		

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PER ABDOMEN

APPEARANCE	NORMAL
VENOUS PROMINENCE	ABSENT
LIVER	NOT PALPABLE
SPLEEN	NOT PALPABLE
HERNIA	ABSENT
ANY OTHER COMMENTS	NIL

CENTRAL NERVOUS SYSTEM

HIGHER FUNCTIONS	NORMAL
CRANIAL NERVES	NORMAL
CEREBELLAR FUNCTIONS	NORMAL
SENSORY SYSTEM	NORMAL
MOTOR SYSTEM	NORMAL
REFLEXES	NORMAL

MUSCULOSKELETAL SYSTEM

SPINE	NORMAL
JOINTS	NORMAL

BASIC EYE EXAMINATION

CONJUNCTIVA	NORMAL
EYELIDS	NORMAL
EYE MOVEMENTS	NORMAL
CORNEA	NORMAL

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Test Report Status **Final**	**Results**	**Biological Reference Interval**	**Units**
DISTANT VISION RIGHT EYE WITHOUT GLASSES	6/6		
DISTANT VISION LEFT EYE WITHOUT GLASSES	6/12		
NEAR VISION RIGHT EYE WITHOUT GLASSES	N/6		
NEAR VISION LEFT EYE WITHOUT GLASSES	N/6		
COLOUR VISION	NORMAL		

BASIC ENT EXAMINATION

EXTERNAL EAR CANAL	NORMAL
TYMPANIC MEMBRANE	NORMAL
NOSE	NO ABNORMALITY DETECTED
SINUSES	NORMAL
THROAT	NORMAL
TONSILS	NOT ENLARGED

BASIC DENTAL EXAMINATION

TEETH	CARIES
GUMS	HEALTHY
ANY OTHER COMMENTS	ADV- RESTORATION.

SUMMARY

RELEVANT HISTORY	NOT SIGNIFICANT
RELEVANT GP EXAMINATION FINDINGS	NOT SIGNIFICANT
RELEVANT LAB INVESTIGATIONS	S. URIC ACID - ABOVE NORMAL LIMITS
RELEVANT NON PATHOLOGY DIAGNOSTICS	USG ABD - ? GB POLYP
REMARKS / RECOMMENDATIONS	MONITOR S. URIC ACID OPHTHALMOLOGIST, SUG. SPL. CONSULTATION DENTAL TREATMENT

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FITNESS STATUS

FITNESS STATUS	FIT (WITH MEDICAL ADVICE) (AS PER REQUESTED PANEL OF TESTS)		

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Test Report Status **Final**	Results	Units

ANNUAL HEALTH CHECK-UP PACKAGE

SCREENING ECHOCARDIOGRAM

CLINICAL PROFILE

ECHO-NORMAL

ULTRASOUND ABDOMEN

ULTRASOUND ABDOMEN

ULTRASOUND WHOLE ABDOMEN

Liver is normal in size, outline & normal echotexture**.** No obvious focal parenchymal lesion/biliary dilatation is seen. Hepatic veins and portal venous radicals are normal.

Gall bladder is well distended and shows a non-mobile tiny echogenic focus without distal shadowing attached to the wall-0 likely polyp. It measures ~1.5-2mm in size. No wall edema is seen.

Pancreas

Pancreas is normal in size, outline and echotexture. No evidence of any focal lesion or calcification is seen.

Pancreatic duct is not dilated.

Spleen

Spleen is normal in size, outline and echotexture .No focal lesion/ calcification is seen.

Kidneys

Both kidneys are normal in size, outline and echotexture. Corticomedullary differentiation is well maintained. Parenchymal thickness is normal. No mass lesion, calculus or hydronephrosis is seen.

No significant retroperitoneal lymphadenopathy/ascites is seen.

Urinary Bladder

Urinary bladder is well distended with normal outline.

Prostate

Prostate is normal in size.

Correlate clinically

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Interpretation(s)
MEDICAL
HISTORY-**********
THIS REPORT CARRIES THE SIGNATURE OF OUR LABORATORY DIRECTOR. THIS IS AN INVIOLABLE FEATURE OF OUR LAB MANAGEMENT SOFTWARE. HOWEVER, ALL EXAMINATIONS AND INVESTIGATIONS HAVE BEEN CONDUCTED BY OUR PANEL OF DOCTORS.

****
FITNESS STATUS-Conclusion on an individual's Fitness, which is commented upon mainly for Pre employment cases, is based on multi factorial findings and does not depend on any one single parameter. The final Fitness assigned to a candidate will depend on the Physician's findings and overall judgement on a case to case basis, details of the candidate's past and personal history as well as the comprehensiveness of the diagnostic panel which has been requested for .These are then further correlated with details of the job under consideration to eventually fit the right man to the right job.
Basis the above, Agilus diagnostic classifies a candidate's Fitness Status into one of the following categories:
• Fit (As per requested panel of tests) – AGILUS Limited gives the individual a clean chit to join the organization, on the basis of the General Physical Examination and the specific test panel requested for.
• Fit (with medical advice) (As per requested panel of tests) - This indicates that although the candidate can be declared as FIT to join the job, minimal problems have been detected during the Pre- employment examination. Examples of conditions which could fall in this category could be cases of mild reversible medical abnormalities such as height weight disproportions, borderline raised Blood Pressure readings, mildly raised Blood sugar and Blood Lipid levels, Hematuria, etc. Most of these relate to sedentary lifestyles and come under the broad category of life style disorders. The idea is to caution an individual to bring about certain lifestyle changes as well as seek a Physician'''''''s consultation and counseling in order to bring back to normal the mildly deranged parameters. For all purposes the individual is FIT to join the job.
• Fitness on Hold (Temporary Unfit) (As per requested panel of tests) - Candidate's reports are kept on hold when either the diagnostic tests or the physical findings reveal the presence of a medical condition which warrants further tests, counseling and/or specialist opinion, on the basis of which a candidate can either be placed into Fit, Fit (With Medical Advice), or Unfit category. Conditions which may fall into this category could be high blood pressure, abnormal ECG, heart murmurs, abnormal vision, grossly elevated blood sugars, etc.
• Unfit (As per requested panel of tests) - An unfit report by Agilus diagnostic Limited clearly indicates that the individual is not suitable for the respective job profile e.g. total color blindness in color related jobs.

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Test Report Status **Final**	Results	Biological Reference Interval	Units

HAEMATOLOGY - CBC

ANNUAL HEALTH CHECK-UP PACKAGE

BLOOD COUNTS, EDTA WHOLE BLOOD

HEMOGLOBIN (HB) METHOD : CYANMETHEMOGLOBIN METHOD	13.7	13.0 - 17.0	g/dL
RED BLOOD CELL (RBC) COUNT METHOD : IMPEDANCE	4.84	4.5 - 5.5	mil/µL
WHITE BLOOD CELL (WBC) COUNT METHOD : CELL COUNTER	8.27	4 - 10	thou/µL
PLATELET COUNT METHOD : IMPEDANCE	245	150 - 410	thou/µL

RBC AND PLATELET INDICES

HEMATOCRIT (PCV) METHOD : CALCULATED	**39.6 Low**	40.0 - 50.0	%
MEAN CORPUSCULAR VOLUME (MCV) METHOD : CELL COUNTER	**81.8 Low**	83.0 - 101.0	fL
MEAN CORPUSCULAR HEMOGLOBIN (MCH) METHOD : CALCULATED PARAMETER	28.4	27.0 - 32.0	pg
MEAN CORPUSCULAR HEMOGLOBIN CONCENTRATION(MCHC) METHOD : CALCULATED PARAMETER	**34.7 High**	31.5 - 34.5	g/dL
RED CELL DISTRIBUTION WIDTH (RDW) METHOD : CALCULATED	**15.0 High**	11.6 - 14.0	%
MEAN PLATELET VOLUME (MPV) METHOD : CALCULATED PARAMETER	10.6	6.8 - 10.9	fL

WBC DIFFERENTIAL COUNT

NEUTROPHILS METHOD : IMPEDANCE / MICROSCOPY	50	40 - 80	%
LYMPHOCYTES METHOD : IMPEDANCE / MICROSCOPY	37	20 - 40	%
MONOCYTES METHOD : IMPEDANCE / MICROSCOPY	5	2.0 - 10.0	%

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EOSINOPHILS METHOD : IMPEDANCE / MICROSCOPY	**8 High**	1 - 6	%
BASOPHILS METHOD : MICROSCOPIC EXAMINATION	0	0 - 2	%
ABSOLUTE NEUTROPHIL COUNT METHOD : CALCULATED PARAMETER	4.14	2 - 7	thou/µL
ABSOLUTE LYMPHOCYTE COUNT METHOD : CALCULATED PARAMETER	**3.06 High**	1.0 - 3.0	thou/µL
ABSOLUTE MONOCYTE COUNT METHOD : CALCULATED PARAMETER	0.41	0.20 - 1.00	thou/µL
ABSOLUTE EOSINOPHIL COUNT METHOD : CALCULATED PARAMETER	**0.66 High**	0.02 - 0.50	thou/µL
ABSOLUTE BASOPHIL COUNT METHOD : CALCULATED PARAMETER	0	0.0 - 0.10	thou/µL

Interpretation(s)
RBC AND PLATELET INDICES-Mentzer index (MCV/RBC) is an automated cell-counter based calculated screen tool to differentiate cases of Iron deficiency anaemia(>13) from Beta thalassaemia trait (<13) in patients with microcytic anaemia. This needs to be interpreted in line with clinical correlation and suspicion. Estimation of HbA2 remains the gold standard for diagnosing a case of beta thalassaemia trait.
WBC DIFFERENTIAL COUNT-The optimal threshold of 3.3 for NLR showed a prognostic possibility of clinical symptoms to change from mild to severe in COVID positive patients. When age = 49.5 years old and NLR = 3.3, 46.1% COVID-19 patients with mild disease might become severe. By contrast, when age < 49.5 years old and NLR < 3.3, COVID-19 patients tend to show mild disease. (Reference to - The diagnostic and predictive role of NLR, d-NLR and PLR in COVID-19 patients A.-P. Yang, et al. International Immunopharmacology 84 (2020) 106504
This ratio element is a calculated parameter and out of NABL scope.

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HAEMATOLOGY

ANNUAL HEALTH CHECK-UP PACKAGE

GLYCOSYLATED HEMOGLOBIN(HBA1C),EDTA WHOLE BLOOD

HBA1C	5.2	Non-diabetic Adult < 5.7 Pre-diabetes 5.7 - 6.4 Diabetes diagnosis: > or = 6.5 Therapeutic goals: < 7.0 Action suggested : > 8.0 (ADA Guideline 2021)	%
METHOD : HPLC			
ESTIMATED AVERAGE GLUCOSE(EAG)	102.5	< 116.0	mg/dL

Interpretation(s)
GLYCOSYLATED HEMOGLOBIN(HBA1C),EDTA WHOLE BLOOD-**Used For**:

1. Evaluating the long-term control of blood glucose concentrations in diabetic patients.
2. Diagnosing diabetes.
3. Identifying patients at increased risk for diabetes (prediabetes).
The ADA recommends measurement of HbA1c (typically 3-4 times per year for type 1 and poorly controlled type 2 diabetic patients, and 2 times per year for well-controlled type 2 diabetic patients) to determine whether a patients metabolic control has remained continuously within the target range.
1. eAG (Estimated average glucose) converts percentage HbA1c to md/dl, to compare blood glucose levels.
2. eAG gives an evaluation of blood glucose levels for the last couple of months.
3. eAG is calculated as eAG (mg/dl) = 28.7 * HbA1c - 46.7

HbA1c Estimation can get affected due to :
1. Shortened Erythrocyte survival : Any condition that shortens erythrocyte survival or decreases mean erythrocyte age (e.g. recovery from acute blood loss,hemolytic anemia) will falsely lower HbA1c test results.Fructosamine is recommended in these patients which indicates diabetes control over 15 days.
2.Vitamin C & E are reported to falsely lower test results.(possibly by inhibiting glycation of hemoglobin.
3. Iron deficiency anemia is reported to increase test results. Hypertriglyceridemia,uremia, hyperbilirubinemia, chronic alcoholism,chronic ingestion of salicylates & opiates addiction are reported to interfere with some assay methods,falsely increasing results.
4. Interference of hemoglobinopathies in HbA1c estimation is seen in

a) Homozygous hemoglobinopathy. Fructosamine is recommended for testing of HbA1c.
b) Heterozygous state detected (D10 is corrected for HbS & HbC trait.)
c) HbF > 25% on alternate paltform (Boronate affinity chromatography) is recommended for testing of HbA1c.Abnormal Hemoglobin electrophoresis (HPLC method) is recommended for detecting a hemoglobinopathy

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BIOCHEMISTRY			
ANNUAL HEALTH CHECK-UP PACKAGE			
GLUCOSE FASTING,FLUORIDE PLASMA			
FBS (FASTING BLOOD SUGAR) METHOD : HEXOKINASE	92	(Normal <100,Impaired fasting glucose:100 to 125,Diabetes mellitus:>=126(on more than 1 occasion)(ADA guidelines 2024)	mg/dL
LIVER FUNCTION PROFILE, SERUM			
BILIRUBIN, TOTAL METHOD : DIAZONIUM ION, BLANKED (ROCHE)	0.49	0.0 - 1.20	mg/dL
BILIRUBIN, DIRECT METHOD : DIAZONIUM ION, BLANKED (ROCHE)	0.20	< or = 0.3	mg/dL
BILIRUBIN, INDIRECT METHOD : CALCULATED PARAMETER	0.29	0.00 - 0.90	mg/dL
TOTAL PROTEIN METHOD : BIURET,SERUM BLANK,ENDPOINT	6.8	6.4 - 8.3	g/dL
ALBUMIN METHOD : BROMOCRESOL PURPLE	4.4	3.97 - 4.94	g/dL
GLOBULIN METHOD : CALCULATED PARAMETER	2.4	2.0 - 4.0	g/dL
ALBUMIN/GLOBULIN RATIO METHOD : CALCULATED PARAMETER	1.8	1.0 - 2.0	RATIO
ASPARTATE AMINOTRANSFERASE(AST/SGOT) METHOD : IFCC WITH PYRIDOXAL 5 PHOSPHATE	17	0 - 40	U/L
ALANINE AMINOTRANSFERASE (ALT/SGPT) METHOD : UV WITH P5P-IFCC	17	0 - 41	U/L
ALKALINE PHOSPHATASE METHOD : PNPP, AMP BUFFER-IFCC	99	40 - 129	U/L
GAMMA GLUTAMYL TRANSFERASE (GGT) METHOD : G-GLUTAMYL-CARBOXY-NITROANILIDE-IFCC	8	8 - 61	U/L
LACTATE DEHYDROGENASE METHOD : L TO P, IFCC	195	135 - 225	U/L

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KIDNEY FUNCTION TEST, SERUM			
BLOOD UREA NITROGEN METHOD : UREASE - UV	10	6 - 20	mg/dL
CREATININE METHOD : ALKALINE PICRATE	0.97	0.90 - 1.30	mg/dL
BUN/CREAT RATIO	10.31	5.00 - 15.00	
URIC ACID METHOD : URICASE, COLORIMETRIC	**7.7 High**	3.4 - 7.0	mg/dL
TOTAL PROTEIN METHOD : BIURET	6.8	6.4 - 8.3	g/dL
ALBUMIN METHOD : BROMOCRESOL PURPLE (BCP) DYE-BINDING	4.4	3.97 - 4.94	g/dL
GLOBULIN METHOD : CALCULATED PARAMETER	2.4	2.0 - 4.0	g/dL
CALCIUM METHOD : BAPTA	9.5	8.4 - 10.2	mg/dL
SODIUM, SERUM METHOD : ISE INDIRECT	142	135 - 145	mmol/L
POTASSIUM, SERUM METHOD : ISE DIRECT	4.41	3.5 - 5.3	mmol/L
CHLORIDE, SERUM METHOD : ISE INDIRECT	103	97 - 110	mmol/L

Interpretation(s)
GLUCOSE FASTING,FLUORIDE PLASMA-**TEST DESCRIPTION**
Normally, the glucose concentration in extracellular fluid is closely regulated so that a source of energy is readily available to tissues and sothat no glucose is excreted in the urine.
Increased in:Diabetes mellitus, Cushing' s syndrome (10 – 15%), chronic pancreatitis (30%). Drugs:corticosteroids,phenytoin, estrogen, thiazides.
Decreased in :Pancreatic islet cell disease with increased insulin,insulinoma,adrenocortical insufficiency,hypopituitarism,diffuse liver disease, malignancy(adrenocortical,stomach,fibrosarcoma),infant of a diabetic mother,enzyme deficiency diseases(e.g.galactosemia),Drugs-insulin,ethanol,propranolol sulfonylureas,tolbutamide,and other oral hypoglycemic agents.
NOTE: While random serum glucose levels correlate with home glucose monitoring results (weekly mean capillary glucose values),there is wide fluctuation within individuals.Thus, glycosylated hemoglobin(HbA1c) levels are favored to monitor glycemic control.
High fasting glucose level in comparison to post prandial glucose level may be seen due to effect of Oral Hypoglycaemics & Insulin treatment,Renal Glyosuria,Glycaemic index & response to food consumed,Alimentary Hypoglycemia,Increased insulin response & sensitivity etc.
LIVER FUNCTION PROFILE, SERUM-
Bilirubin is a yellowish pigment found in bile and is a breakdown product of normal heme catabolism. Bilirubin is excreted in bile and urine, and elevated levels may give yellow discoloration in jaundice.**Elevated levels** results from increased bilirubin production (eg, hemolysis and ineffective erythropoiesis), decreased bilirubin excretion (eg, obstruction and hepatitis), and abnormal bilirubin metabolism (eg, hereditary and neonatal jaundice). Conjugated (direct) bilirubin is elevated more than unconjugated

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(indirect) bilirubin in Viral hepatitis, Drug reactions, Alcoholic liver disease Conjugated (direct) bilirubin is also elevated more than unconjugated (indirect) bilirubin when there is some kind of blockage of the bile ducts like in Gallstones getting into the bile ducts, tumors &Scarring of the bile ducts. Increased unconjugated (indirect) bilirubin may be a result of Hemolytic or pernicious anemia, Transfusion reaction & a common metabolic condition termed Gilbert syndrome, due to low levels of the enzyme that attaches sugar molecules to bilirubin.

AST is an enzyme found in various parts of the body. AST is found in the liver, heart, skeletal muscle, kidneys, brain, and red blood cells, and it is commonly measured clinically as a marker for liver health. AST levels increase during chronic viral hepatitis, blockage of the bile duct, cirrhosis of the liver,liver cancer,kidney failure,hemolytic anemia,pancreatitis,hemochromatosis. AST levels may also increase after a heart attack or strenuous activity.ALT test measures the amount of this enzyme in the blood.ALT is found mainly in the liver, but also in smaller amounts in the kidneys,heart,muscles, and pancreas.It is commonly measured as a part of a diagnostic evaluation of hepatocellular injury, to determine liver health.AST levels increase during acute hepatitis,sometimes due to a viral infection,ischemia to the liver,chronic hepatitis,obstruction of bile ducts,cirrhosis.

ALP is a protein found in almost all body tissues.Tissues with higher amounts of ALP include the liver,bile ducts and bone.Elevated ALP levels are seen in Biliary obstruction, Osteoblastic bone tumors, osteomalacia, hepatitis, Hyperparathyroidism, Leukemia, Lymphoma, Pagets disease,Rickets,Sarcoidosis etc. Lower-than-normal ALP levels seen in Hypophosphatasia,Malnutrition,Protein deficiency,Wilsons disease.

GGT is an enzyme found in cell membranes of many tissues mainly in the liver,kidney and pancreas.It is also found in other tissues including intestine,spleen,heart, brain and seminal vesicles.The highest concentration is in the kidney,but the liver is considered the source of normal enzyme activity.Serum GGT has been widely used as an index of liver dysfunction.Elevated serum GGT activity can be found in diseases of the liver,biliary system and pancreas.Conditions that increase serum GGT are obstructive liver disease,high alcohol consumption and use of enzyme-inducing drugs etc.

Total Protein also known as total protein,is a biochemical test for measuring the total amount of protein in serum.Protein in the plasma is made up of albumin and globulin.Higher-than-normal levels may be due to:Chronic inflammation or infection,including HIV and hepatitis B or C,Multiple myeloma,Waldenstroms disease.Lower-than-normal levels may be due to: Agammaglobulinemia,Bleeding (hemorrhage),Burns,Glomerulonephritis,Liver disease, Malabsorption,Malnutrition,Nephrotic syndrome,Protein-losing enteropathy etc.

Albumin is the most abundant protein in human blood plasma.It is produced in the liver.Albumin constitutes about half of the blood serum protein.Low blood albumin levels (hypoalbuminemia) can be caused by:Liver disease like cirrhosis of the liver, nephrotic syndrome,protein-losing enteropathy,Burns,hemodilution,increased vascular permeability or decreased lymphatic clearance,malnutrition and wasting etc

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CODE/NAME & ADDRESS : C000133041 CHECKMED PRIVATE LIMITED SECOND FLOOR, SCO 334-334, SECTOR 35,CHANDIGARH CHANDIGARH 160022 9914256267	ACCESSION NO : **0062YH000953** PATIENT ID : KUNAM08119462 CLIENT PATIENT ID: ABHA NO :	AGE/SEX : 30 Years Male DRAWN : RECEIVED : 14/08/2025 09:11:43 REPORTED : 16/08/2025 13:01:42

Test Report Status **Final**	Results	Biological Reference Interval	Units
BIOCHEMISTRY - LIPID			
ANNUAL HEALTH CHECK-UP PACKAGE			
LIPID PROFILE WITH CALCULATED LDL,SERUM			
CHOLESTEROL, TOTAL	179	< 200 Desirable 200 - 239 Borderline High >/= 240 High	mg/dL
METHOD : CHOLESTEROL OXIDASE, ESTERASE,PEROXIDASE			
TRIGLYCERIDES	61	< 150 Normal 150 - 199 Borderline High 200 - 499 High >/=500 Very High	mg/dL
METHOD : ENZYMATIC, END POINT			
HDL CHOLESTEROL	51	< 40 Low >/=60 High	mg/dL
METHOD : DIRECT MEASURE POLYMER-POLYANION			
CHOLESTEROL LDL	**116 High**	< 100 Optimal 100 - 129 Near optimal/ above optimal 130 - 159 Borderline High 160 - 189 High >/= 190 Very High	mg/dL
METHOD : CALCULATED			
NON HDL CHOLESTEROL	128	Desirable-Less than 130 Above Desirable-130-159 Borderline High-160-189 High-190-219 Very High- >or =220	mg/dL
METHOD : CALCULATED			
VERY LOW DENSITY LIPOPROTEIN	12.2	< or = 30	mg/dL
METHOD : CALCULATED			
CHOL/HDL RATIO	3.5	3.3 - 4.4: Low Risk 4.5 - 7.0: Average Risk 7.1 - 11.0: Moderate Risk >11.0: High Risk	
METHOD : CALCULATED			
LDL/HDL RATIO	2.3	0.5 - 3.0 Desirable/Low Risk 3.1 - 6.0 Borderline/Moderate Risk >6.0 High Risk	
METHOD : CALCULATED			

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Interpretation(s)

Serum lipid profile is measured for cardiovascular risk prediction. Lipid Association of India recommends LDL-C as primary target and Non HDL-C as co-primary treatment target.

Risk Stratification for ASCVD (Atherosclerotic cardiovascular disease) by Lipid Association of India

Risk Category	
Extreme risk group	A.CAD with > 1 feature of high risk group
	B. CAD with > 1 feature of Very high risk group or recurrent ACS (within 1 year) despite LDL-C < or = 50 mg/dl or polyvascular disease
Very High Risk	1. Established ASCVD 2. Diabetes with 2 major risk factors or evidence of end organ damage 3. Familial Homozygous Hypercholesterolemia
High Risk	1. Three major ASCVD risk factors. 2. Diabetes with 1 major risk factor or no evidence of end organ damage. 3. CKD stage 3B or 4. 4. LDL >190 mg/dl 5. Extreme of a single risk factor. 6. Coronary Artery Calcium - CAC >300 AU. 7. Lipoprotein a >/= 50mg/dl 8. Non stenotic carotid plaque
Moderate Risk	2 major ASCVD risk factors
Low Risk	0-1 major ASCVD risk factors

Major ASCVD (Atherosclerotic cardiovascular disease) Risk Factors	
1. Age > or = 45 years in males and > or = 55 years in females	3. Current Cigarette smoking or tobacco use
2. Family history of premature ASCVD	4. High blood pressure
5. Low HDL	

Newer treatment goals and statin initiation thresholds based on the risk categories proposed by LAI in 2020.

Risk Group	**Treatment Goals**		**Consider Drug Therapy**	
	LDL-C (mg/dl)	**Non-HDL (mg/dl)**	**LDL-C (mg/dl)**	**Non-HDL (mg/dl)**
Extreme Risk Group Category A	<50 (Optional goal < OR = 30)	< 80 (Optional goal <OR = 60)	>OR = 50	>OR = 80
Extreme Risk Group Category B	<OR = 30	<OR = 60	> 30	>60
Very High Risk	<50	<80	>OR= 50	>OR= 80
High Risk	<70	<100	>OR= 70	>OR= 100
Moderate Risk	<100	<130	>OR= 100	>OR= 130
Low Risk	<100	<130	>OR= 130*	>OR= 160

*After an adequate non-pharmacological intervention for at least 3 months.

References: Management of Dyslipidaemia for the Prevention of Stroke: Clinical Practice Recommendations from the Lipid Association of India. Current Vascular Pharmacology, 2022, 20, 134-155.

Interpretation of Diagnostic Tests by Jacques Wallach, 11th Ed.

Friedewald, W. T., Levy, R. I., & Fredrickson, D. S. (1972). "Estimation of the concentration of low-density lipoprotein cholesterol in plasma, without use of the preparative ultracentrifuge."Clinical Chemistry, 18(6), 499-502.

National Cholesterol Education Program (NCEP) Adult Treatment Panel III (ATP III). (2001).

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Third Report of the National Cholesterol Education Program (NCEP) Expert Panel on Detection, Evaluation, and Treatment of High Blood Cholesterol in Adults (Adult Treatment Panel III) Final Report. Circulation, 106(25), 3143-3421.

American College of Cardiology (ACC) and American Heart Association (AHA) (2013). 2013 ACC/AHA Guideline on the Assessment of Cardiovascular Risk. Circulation, 129(25), S49-S73.

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Test Report Status **Final**	Results	Biological Reference Interval	Units
NEPHELOMETRY			
ANNUAL HEALTH CHECK-UP PACKAGE			
C-REACTIVE PROTEIN, SERUM (QUANTITATIVE)			
C-REACTIVE PROTEIN	3.3	< 5.0	mg/L
METHOD : PARTICLE ENHANCED IMMUNOTURBIDIMETRIC ASSAY			

Interpretation(s)

C-REACTIVE PROTEIN, SERUM (QUANTITATIVE)- **Test Description:**

A CRP test measures the amount of CRP in the blood to detect inflammation due to acute conditions or to monitor the severity of disease in chronic conditions. CRP is one of the proteins commonly referred to as acute phase reactants. CRP is distinguished by its rapid response to trauma or infection.Synthesis of CRP increases within 4-6 hours of onset of inflammation, reaching peak values within 1-2 days. CRP levels also fall quickly after resolution of inflammation since its half life is 6 hours.

This standard CRP test is not to be confused with a hs-CRP test. These are two different tests that measure CRP and each test measures a different range of CRP levels in the blood for different purposes. The standard CRP test measures high levels of protein observed in diseases that cause significant inflammation.

Test Interpretation:

Increased CRP level: Increasing amount of CRP in the blood suggests the presence of inflammation but will not identify its location or the cause.

Suspected bacterial infection: a high CRP level can confirm that you have a serious bacterial infection.

Chronic inflammatory disease: high levels of CRP suggest a flare-up if you have a chronic inflammatory disease or that treatment has not been effective.

Testing for CRP is indicated in the following clinical situations - monitoring recovery from surgery, myocardial infarction, transplantation, inflammatory bowel disease, rheumatic diseases and infectious diseases. Measuring and charting C-reactive protein values can also prove useful in determining disease progress or the effectiveness of treatments

CRP levels can be elevated in the later stages of pregnancy as well as with the use of birth control pills or hormone replacement therapy (i.e., estrogen). Higher levels of CRP have also been observed in people who are obese. CRP can also be increased in people who have cancer.

Recommendation: The hs-CRP test precisely detects lower levels of the protein than that measured by the standard CRP test and is also used to evaluate individuals for risk of cardiovascular disease. It measures CRP in the range from 0.15 to 20 mg/L.

Limitation:

CRP levels in autoimmune diseases may show little or no increase unless infection is present. Levels may not increase in conditions like pregnancy, angina, seizures, asthma, common cold. The main limitation of CRP is in its non-specific response and should not be interpreted without a complete clinical history and evaluation.

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Test Report Status **Final**	**Results**	**Biological Reference Interval**	**Units**
SEROLOGY			
ANNUAL HEALTH CHECK-UP PACKAGE			
VDRL,SERUM			
VDRL (MODIFIED)	NON REACTIVE	NONREACTIVE	TITER

Interpretation(s)
VDRL,SERUM-**Test description:-**
VDRL/RPR/TRUST are Non-treponemal screening tests for syphilis.They achieve a sensitivity of 100% in the detection of secondary syphilis.After the secondary stage, titres dimish and about a third of patients with late syphilis are seronegative.The titres may rise in patients developing cardiovascular, neurological or gummatous lesions. The tests usually become negative 6 to 18 months after effective treatment of syphilis, depending on the stage at which treatment is intitiated.If treatment is started late, tests may remain positive in low titres.

Interpretation:-
Diagnostic titre for VDRL is >=1:8. All Titres < 1:8 must be confirmed with a treponemal test (TPHA) for diagnosing syphilis,as it may be a biological false positive result.

Limitations:-
Nontreponemal tests lack sensitivity in primary and late syphilis.False negative reactions can occur in stages of the disease where there is minimal tissue damage, particularly in early infection and in latent stages.
Biologic false positive reactions are common in a variety of other infections (hepatitis, Leprosy,Malaria,Relapsing fever,Infectious mononucleosis,hepatits), Rheumatic diseases and Auto- Immune disorders and may persist upto 6 months. Chronic false-positive reactions (persisting beyond 6 months) may be seen in connective tissue diseases ,immunoglobulin abnormalities, narcotic addiction, leprosy, and malignancy.
More specific Treponemal tests, such as Treponema pallidum Hemagglutination assay (TPHA) test are recommended for confirmation.

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CLINICAL PATH - URINALYSIS

ANNUAL HEALTH CHECK-UP PACKAGE

PHYSICAL EXAMINATION, URINE

COLOR METHOD : VISUAL EXAMINATION	PALE YELLOW		
APPEARANCE METHOD : VISUAL EXAMINATION	CLEAR		

CHEMICAL EXAMINATION, URINE

PH METHOD : PH INDICATOR AND REFLECTANCE, SPECTROPHOTOMETRY	6.0	4.5 - 7.5	
SPECIFIC GRAVITY METHOD : PKA CHANGE WITH REFLECTANCE, SPECTROPHOTOMETRY	1.005	1.005 - 1.030	
PROTEIN METHOD : PROTEIN ERROR OF INDICATORS WITH REFLECTANCE, SPECTROPHOTOMETRY	NOT DETECTED	NOT DETECTED	
GLUCOSE METHOD : GLUCOSE OXIDASE WITH REFLECTANCE, SPECTROPHOTOMETRY	NOT DETECTED	NEGATIVE	
KETONES METHOD : ROTHERA'S WITH REFLECTANCE, SPECTROPHOTOMETRY	NOT DETECTED	NOT DETECTED	
BLOOD METHOD : PEROXIDASE METHOD WITH REFLECTANCE, SPECTROPHOTOMETRY	NOT DETECTED	NEGATIVE	
BILIRUBIN METHOD : DIAZOTIZED WITH REFLECTANCE, SPECTROPHOTOMETRY	NOT DETECTED	NOT DETECTED	
UROBILINOGEN METHOD : EHRLICH REACTION WITH REFLECTANCE, SPECTROPHOTOMETRY	NORMAL	NORMAL	
NITRITE METHOD : DIAZONIUM COMPOUND WITH REFLECTANCE, SPECTROPHOTOMETRY	NOT DETECTED	NOT DETECTED	
LEUKOCYTE ESTERASE METHOD : ESTERASES METHOD WITH REFLECTANCE, SPECTROPHOTOMETRY	NOT DETECTED	NOT DETECTED	

MICROSCOPIC EXAMINATION, URINE

RED BLOOD CELLS	NOT DETECTED	0 - 2	/HPF
PUS CELL (WBCS) METHOD : MICROSCOPY	0-1	0-5	/HPF

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Test Report Status **Final**	Results	Biological Reference Interval	Units
EPITHELIAL CELLS METHOD : MICROSCOPY	0-1	0-5	/HPF
CASTS METHOD : MICROSCOPY	NOT DETECTED		
CRYSTALS METHOD : MICROSCOPY	NOT DETECTED		
BACTERIA METHOD : MICROSCOPY	NOT DETECTED	NOT DETECTED	
YEAST METHOD : MICROSCOPY	NOT DETECTED	NOT DETECTED	

Interpretation(s)

The following table describes the probable conditions, in which the analytes are present in urine

Presence of	Conditions
Proteins	Inflammation or immune illnesses
Pus (White Blood Cells)	Urinary tract infection, urinary tract or kidney stone, tumors or any kind of kidney impairment
Glucose	Diabetes or kidney disease
Ketones	Diabetic ketoacidosis (DKA), starvation or thirst
Urobilinogen	Liver disease such as hepatitis or cirrhosis
Blood	Renal or genital disorders/trauma
Bilirubin	Liver disease
Erythrocytes	Urological diseases (e.g. kidney and bladder cancer, urolithiasis), urinary tract infection and glomerular diseases
Leukocytes	Urinary tract infection, glomerulonephritis, interstitial nephritis either acute or chronic, polycystic kidney disease, urolithiasis, contamination by genital secretions
Epithelial cells	Urolithiasis, bladder carcinoma or hydronephrosis, ureteric stents or bladder catheters for prolonged periods of time
Granular Casts	Low intratubular pH, high urine osmolality and sodium concentration, interaction with Bence-Jones protein
Hyaline casts	Physical stress, fever, dehydration, acute congestive heart failure, renal diseases

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Calcium oxalate	Metabolic stone disease, primary or secondary hyperoxaluria, intravenous infusion of large doses of vitamin C, the use of vasodilator naftidrofuryl oxalate or the gastrointestinal lipase inhibitor orlistat, ingestion of ethylene glycol or of star fruit (Averrhoa carambola) or its juice
Uric acid	arthritis
Bacteria	Urinary infectionwhen present in significant numbers & with pus cells.
Trichomonas vaginalis	Vaginitis, cervicitis or salpingitis

Reference:

Henrys Clinical Diagnostics and Management by Laboratory Methods 21st edition (pg 410).

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SPECIALISED CHEMISTRY - HORMONE

ANNUAL HEALTH CHECK-UP PACKAGE

THYROID PANEL, SERUM

T3 METHOD : ECLIA	107.50	80.0 - 200.0	ng/dL
T4 METHOD : ECLIA	7.81	5.10 - 14.10	µg/dL
TSH (ULTRASENSITIVE) METHOD : ECLIA	3.130	0.270 - 4.200	µIU/mL

Interpretation(s)

Triiodothyronine T3 , **Thyroxine T4,** and **Thyroid Stimulating Hormone TSH** are thyroid hormones which affect almost every physiological process in the body, including growth, development, metabolism, body temperature, and heart rate.
Production of T3 and its prohormone thyroxine (T4) is activated by thyroid-stimulating hormone (TSH), which is released from the pituitary gland. Elevated concentrations of T3, and T4 in the blood inhibit the production of TSH.
Excessive secretion of thyroxine in the body is hyperthyroidism, and deficient secretion is called hypothyroidism.
In primary hypothyroidism, TSH levels are significantly elevated, while in secondary and tertiary hyperthyroidism, TSH levels are low.
Below mentioned are the guidelines for Pregnancy related reference ranges for Total T4, TSH & Total T3.Measurement of the serum TT3 level is a more sensitive test for the diagnosis of hyperthyroidism, and measurement of TT4 is more useful in the diagnosis of hypothyroidism.Most of the thyroid hormone in blood is bound to transport proteins. Only a very small fraction of the circulating hormone is free and biologically active. It is advisable to detect Free T3, FreeT4 along with TSH, instead of testing for albumin bound Total T3, Total T4**.**

Sr. No.	TSH	Total T4	FT4	Total T3	Possible Conditions
1	High	Low	Low	Low	(1) Primary Hypothyroidism (2) Chronic autoimmune Thyroiditis (3) Post Thyroidectomy (4) Post Radio-Iodine treatment
2	High	Normal	Normal	Normal	(1)Subclinical Hypothyroidism (2) Patient with insufficient thyroid hormone replacement therapy (3) In cases of Autoimmune/Hashimoto thyroiditis (4). Isolated increase in TSH levels can be due to Subclinical inflammation, drugs like amphetamines, Iodine containing drug and dopamine antagonist e.g. domperidone and other physiological reasons.
3	Normal/Low	Low	Low	Low	(1) Secondary and Tertiary Hypothyroidism
4	Low	High	High	High	(1) Primary Hyperthyroidism (Graves Disease) (2) Multinodular Goitre (3)Toxic Nodular Goitre (4) Thyroiditis (5) Over treatment of thyroid hormone (6) Drug effect e.g. Glucocorticoids, dopamine, T4 replacement therapy (7) First trimester of Pregnancy
5	Low	Normal	Normal	Normal	(1) Subclinical Hyperthyroidism

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6	High	High	High	High	(1) TSH secreting pituitary adenoma (2) TRH secreting tumor
7	Low	Low	Low	Low	(1) Central Hypothyroidism (2) Euthyroid sick syndrome (3) Recent treatment for Hyperthyroidism
8	Normal/Low	Normal	Normal	High	(1) T3 thyrotoxicosis (2) Non-Thyroidal illness
9	Low	High	High	Normal	(1) T4 Ingestion (2) Thyroiditis (3) Interfering Anti TPO antibodies

REF: 1. TIETZ Fundamentals of Clinical chemistry 2.Guidlines of the American Thyroid association duriing pregnancy and Postpartum, 2011.

TSH in pregnancy

There's reduction in both the lower and the upper limit of maternal TSH relative to the non-pregnant TSH reference range. This is because of elevated levels of serum hCG that directly stimulates the TSH receptor, thereby increasing thyroid hormone production. The largest decrease in serum TSH is observed during the first trimester. Thereafter, serum TSH and its reference range gradually increases in the second and third trimesters, but nonetheless remains lower than in non-pregnant women.

NOTE: It is advisable to detect Free T3,FreeT4 along with TSH, instead of testing for albumin bound Total T3, Total T4.TSH is not affected by variation in thyroid - binding protein. TSH has a diurnal rhythm, with peaks at 2:00 - 4:00 a.m. And troughs at 5:00 - 6:00 p.m. With ultradian variations.

****End Of Report****

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